INDIVIDUAL CASE SAFETY REPORT (ICSR)

Narrative, clinical assessment and MedDRA term mapping

Case reference: SYN-PV-015 **Status:** Draft — follow-up required

Source: user-provided classification_cases worksheet screenshot and case details already supplied in this conversation. Truncated or absent fields are not guessed.

1. Case summary

Field	Recorded information
Patient	33-year-old male
Suspect drug	Omeprazole
Dose / regimen	20 mg once daily; oral (dose/regimen confirmed from case details; worksheet text is clipped)
Indication	Hypertension (as recorded; verify indication)
Reaction	Headache
Onset	16 days after treatment initiation
Expectedness	Known (source field; reference safety information not supplied)
Seriousness	Hospitalisation (source field; admission details not supplied)
Dechallenge	Unknown
Rechallenge	Positive (supporting dates/details not supplied)
Concomitant medicines	None reported
Reporter	Physician; name/contact details not supplied

2. MedDRA term mapping

Verbatim/source term	MedDRA level	Proposed term
Headache	Preferred Term (PT)	**Headache**
Headache	System Organ Class (SOC)	**Nervous system disorders**

Coding limitation: The full verbatim reporter wording and MedDRA version/Browser were not supplied. PT and SOC are proposed based on the recorded term. No LLT or numeric code is asserted; verify in the licensed MedDRA version and document the version/date.

3. Clinical assessment

Chronology: Headache onset was recorded 16 days after omeprazole initiation. This establishes temporal sequence, not causality.

Seriousness: The dataset says “Hospitalisation”. Confirm whether inpatient hospitalisation occurred or was prolonged; obtain admission/discharge dates, reason and supporting clinical details.

Severity: Not reported. Do not infer severity from the seriousness classification; they are different concepts.

Expectedness: “Known” is the dataset value. Independent expectedness assessment requires the applicable reference safety information, product and version/date.

Dechallenge: Recorded as “Unknown”; no positive dechallenge conclusion can be made.

Rechallenge: Recorded as “Positive”. Clarify whether headache recurred after re-administration, with restart date, dose, time to recurrence and outcome.

Alternative causes: Not assessable; medical history, intercurrent illness and other possible causes were not provided.

Causality: A final causality conclusion is not supportable from the available data. Do not classify as probable/definite based only on these fields.

4. Draft ICSR narrative

A 33-year-old male patient was reported to have experienced headache during treatment with omeprazole 20 mg once daily administered orally. Omeprazole was recorded as being used for hypertension in the source worksheet. Headache was reported to have started 16 days after treatment initiation. The event was classified as “Hospitalisation” in the dataset; however, details confirming the admission, its dates, duration and clinical reason were not available. Expectedness was recorded as “Known”, but the applicable reference safety information was not supplied for independent verification. Dechallenge was recorded as “Unknown” and rechallenge as “Positive”; the underlying dates and clinical description supporting these assessments were not supplied. No concomitant medicines were reported. The reporter was identified as a physician; reporter name and contact details were unavailable. The reaction term “Headache” is provisionally mapped to MedDRA PT “Headache” under SOC “Nervous system disorders”. Based on the information currently available, a final causality assessment cannot be established and additional information is required.

5. Follow-up queries / data gaps

• Obtain reporter name, contact details, country and report receipt date; confirm reporter identifiability.

• Obtain the full case narrative, exact event description, event course, outcome and event end date.

• Confirm actual hospitalisation, admission/discharge dates, reason and whether the event caused or prolonged inpatient admission.

• Clarify rechallenge: was omeprazole restarted and did headache recur? Obtain dates, dose and time to recurrence.

• Clarify dechallenge and whether headache improved after stopping omeprazole.

• Verify the recorded indication “hypertension”.

• Obtain relevant medical history, alternative causes, investigations and treatment for headache.

• Confirm product/brand, manufacturer, start/stop dates and any dose changes.

• Verify expectedness against applicable reference safety information; document MedDRA version and coding review.

6. Validity and audit disposition

Disposition: Draft — follow-up required. The four minimum ICSR elements are an identifiable patient, identifiable reporter, suspected medicinal product and suspected adverse reaction. Patient age/sex, suspect drug and reaction are recorded. “Physician” alone does not establish an identifiable reporter; reporter details are required before the case can be confirmed valid.

Audit trail: Preserve original source values; document each follow-up attempt, date, source and response; retain traceability for corrections; record coder/reviewer, MedDRA version and coding date; document medical review and seriousness/expectedness rationale.

7. Structured ICSR sections

Patient: 33-year-old male. Other history and additional patient details not provided.

Reaction: Headache; onset day 16; seriousness field: Hospitalisation; outcome/end date not provided. Proposed PT: Headache; SOC: Nervous system disorders.

Suspect drug: Omeprazole 20 mg once daily, oral; indication recorded as hypertension. Product details and treatment dates not provided.

Reporter: Physician. Name, contact details, country and report receipt date not provided.

Training simulation only. This document is not a validated safety database output, medical advice or regulatory submission. Unresolved fields must remain unknown until verified.